

KET QUA SINH HOA MAU



Patient:	MUN	Species:	Canine	Patient ID:	
Client:	HUY	Gender:	Male	Sample No.:	106
Doctor:		Age:	Adult 4Y	Time of analysis:	2026/07/02 09:33

Item			Current result		Ref. Ranges	
Protein	TP	H-	73.1	g/L	52.0-82.0	
Protein	ALB		34.8	g/L	21.0-40.0	
Protein	GLOB		38.4	g/L	25.4-52.0	
Protein	A/G		0.9			
Liver and gallbladder	ALT		41.1	U/L	10.1-100.3	
Liver and gallbladder	AST	H+	42.0	U/L	0.0-51.7	
Liver and gallbladder	AST/ALT		1.02			
Liver and gallbladder	ALP	↓	13.0	U/L	15.5-212.0	
Liver and gallbladder	GGT		<2.0	U/L	0.0-15.9	
Liver and gallbladder	TBIL	H-	<1.70	μmol/L	0.00-15.00	
Liver and gallbladder	TBA		2.6	μmol/L	0.0-30.0	
Pancreas	AMY		596.1	U/L	397.7-1285.1	
Kidneys	BUN	↑	10.89	mmol/L	2.50-9.77	
Kidneys	CREA		65.20	μmol/L	20.00-123.70	
Kidneys	BUN/CREA		41.3			
Cardiovasc./Muscle	CK		222.9	U/L	66.4-257.5	
Cardiovasc./Muscle	LDH	↑ H+	234.9	U/L	0.0-143.6	
Energy metabolism	GLU	↓	2.28	mmol/L	3.80-7.50	
Energy metabolism	TC	↑ H+	9.17	mmol/L	2.67-8.38	
Energy metabolism	TG	↑	2.32	mmol/L	0.10-1.30	
Minerals	Ca		2.55	mmol/L	2.10-2.97	
Minerals	PHOS	↓	0.68	mmol/L	0.70-2.20	
Minerals	CaxP		1.74	(mmol/L) ²		
Minerals	Mg		0.65	mmol/L	0.53-1.06	
Electrolytes	Na+		146.1	mmol/L	138.0-160.0	
Electrolytes	K+	↓ H+	3.4	mmol/L	3.5-5.9	
Electrolytes	Na/K		43.0			
Electrolytes	Cl-	↓	99.6	mmol/L	100.7-125.0	

Operator:

Comprehensive Diagnosis Panel

QC QC OK

HEM(Hemolysis degree): 2+ LIP(Lipemia degree): 1+ ICT(Jaundice degree): 0



Report Explen.

ALP



Increase is commonly associated with fracture healing period, hepatobiliary diseases, hyperthyroidism, and osteosarcoma, etc.

The results only applies to this test sample.

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Report Explan.

BUN



Increase is commonly associated with high protein diet, gastrointestinal bleeding, nephropathy, and urinary obstruction, etc. Reduction is commonly associated with insufficient protein intake and liver failure, etc.

LDH



Increase is commonly associated with hemolysis (especially in canine), post-exercise, liver injury, exertional rhabdomyolysis, white muscle disease, myocardial injury, tumors, etc.

GLU



Increase is commonly associated with diabetes and hypercorticism, etc. Reduction is commonly associated with insulin administration, malnutrition, and insulinoma, etc.

TC



Increase is commonly associated with biliary obstruction, hypothyroidism, hypercorticism, nephropathy, diabetes, etc. Reduction is commonly associated with protein loss enteropathy, pancreatic exocrine insufficiency, and hypoadrenocorticism, etc.

TG



Increase is commonly associated with postprandial, obesity, diabetes and hypercorticism, etc.

PHOS



Increase is commonly associated with nephropathy, bone healing period, and hyperthyroidism. Decreased in hyperparathyroidism, tumor, etc.

K+



Increase is commonly associated with high potassium fluid replacement, diabetes, adrenocortical hypofunction, and acute kidney injury, etc. Reduction is commonly associated with low potassium or potassium-free fluid replacement, vomiting, diarrhea, and hypercorticism, etc.

Cl-



Increase is commonly associated with salt intoxication, hypertonic NaCl solution rehydration, small intestinal diarrhea, etc. Reduction is commonly associated with vomiting, diuretic therapy, etc.

Note: Due to the complexity and individuality of disease diagnosis, the report interpretation is only for your reference. Please consult your doctors for clinical diagnosis results.
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